

Preface

The United States, boasting the world's largest GDP and ranking ninth in terms of per capita GDP, allocates a staggering 18% of its GDP to the healthcare system. However, the current state of the healthcare system is a subject of debate. According to a 2022 *Gallup* survey, 48% of respondents perceive the quality of healthcare as good, while the remaining portion rates it as fair or poor.

With such substantial spending, one might naturally wonder: Why, despite this financial commitment, does the system fail to meet the expectations of the public? This question becomes even more pressing when we consider the challenges and disparities that persist within the U.S. healthcare system. Issues related to underinsured, increasing healthcare costs, and lack of insurance coverage continue to trouble the nation (*KFF Survey*). In exploring these complexities, we aim to shed light on the intricacies of the American healthcare system and uncover potential plans for improvement.

A Historical Overview of the U.S. Healthcare System

To understand what happened, also what historical events build the health system today. It is better if we walk through the history.

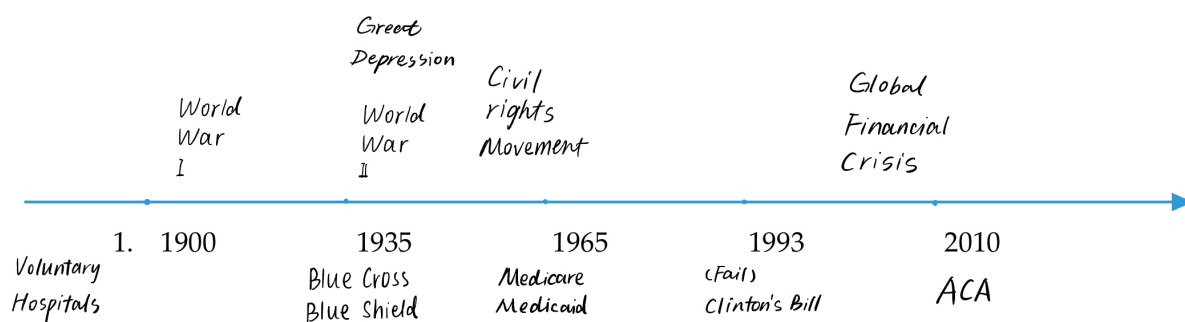


Figure.1 Historical Events and Health Care Reform

Pre-1945: Foundations of American Healthcare

At the outset of World War I, while Western European nations embraced government-supported communal institutions for achieving social goals through sickness insurance programs, the United States took a distinct path, blending social insurance and state backing. European counterparts repurposed existing communal risk pools into state-backed endeavors. However, the challenges faced by public health insurance in the U.S. cannot be attributed solely to deep-seated suspicions on government and the preference for voluntary social provision.

The legislative failure can also be attributed to the limited authority of the federal government, which often faces resistance from private businesses when proposing new taxes or regulations. Furthermore, the government is concerned about the potential negative impact of imposing tax or mandate policies on private investments, which could potentially weaken market dynamics.

In the 1910s, Theodore Roosevelt's attempts at healthcare reform failed, leading the American Association for Labor Legislation (AALL) to take the initiative at the state legislative level. In 1916, the AALL lobbied for the adoption of a German-style healthcare system, aiming to enhance worker security and promote preventive measures. However, despite its influence in shaping state political agendas, the AALL faced challenges in passing proposals that raised concerns among businesses.

During the 1920s, the effort of AALL failed, as it garnered opposition from groups like the American Medical Association (AMA), which strongly opposed government involvement in healthcare insurance. The AMA actively worked against the Sheppard-Towner Act and feared that increased government intervention could erode physician autonomy. This caution extended to their approach to voluntary insurance plans, as they were concerned about losing control over fees and treatment due to third-party involvement (*Hacker, p. 198*).

It is noteworthy that the AMA's ascent can be attributed to the increased income of physicians during and after the war, as detailed by Hacker (*p. 197*). The group insurance even increased during the Great Depression. This underscores the significant influence of economic factors on the evolution of healthcare systems. Moreover, it serves as a contemporary reminder of the challenges faced when attempting to enact drug price regulation legislation today.

In 1929, Blue Cross was founded, initially as a pure non-profit organization, with the support of the American Hospital Association (AHA) and other funds. It began as single-hospital plans and expanded to community-wide arrangements (*Hacker, p. 203*). Blue Cross primarily involved direct payment to medical providers and was initially limited to hospital coverage, avoiding resistance from physicians. Over time, as private insurance options developed and competed with Blue Cross, the organization gradually adopted more business-like characteristics.

During the 1930s, a transformative period within the New Deal Era, the United States witnessed significant shifts in its approach to social protection. While the era marked the implementation of the Social Security Act, which addressed critical aspects of social welfare, including old-age and unemployment concerns, the pursuit of comprehensive general healthcare was temporarily overshadowed. This period of transition laid the foundational concepts for future healthcare programs like Medicare and Medicaid. Concurrently, private insurance gained prominence, although it was not the primary impediment to government-led health insurance (*Hacker, p.220*). Proponents of private health insurance emphasized the importance of voluntary initiatives in public policy, questioning the necessity for extensive government-driven social insurance within the healthcare sector.

Post-1945: Evolving Healthcare Landscape

Public Insurance: Medicaid and Medicare

Against the background of a predominantly private insurance-dominated market, the state introduced two significant bills aimed at expanding coverage to vulnerable populations, including the elderly and low-income individuals. Prior to the implementation of these programs, these groups often relied on charity care for assistance.

Here I want to show a more clear map of these two public option: Medicare and Medicaid have significantly improved healthcare access in the United States, but several gaps persist. Undocumented immigrants often face barriers to affordable care due to ineligibility for these programs. Primary care shortages, particularly in rural areas, can hinder accessibility for Medicare and Medicaid patients. High prescription drug costs and the coverage gap challenge some Medicare beneficiaries. Additionally, low-income adults in states that haven't expanded Medicaid may find themselves in a coverage gap without access to subsidized insurance. (*How many uninsured are in the coverage gap, KFF*)

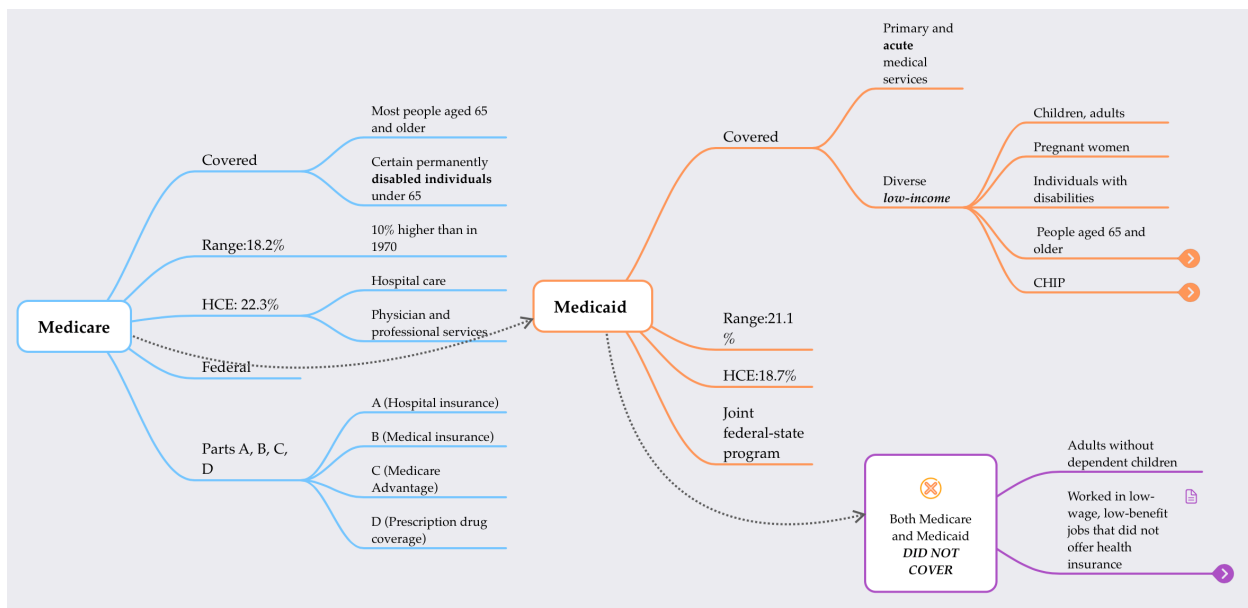


Figure.2 Comparison of Medicare And Medicaid

Clinton's Reform

President Clinton's healthcare reform plan aimed to regulate private insurance companies and control healthcare costs without imposing new taxes. It sought to make healthcare more affordable, increase choices for consumers, and improve the quality of services.

While this proposal was undoubtedly ambitious, it faced challenges and ultimately failed to gain approval. One reason for its downfall was its approach to premium regulation, which included setting price ceilings. This move had significant implications for private insurers and may have seemed like government overreach. It's essential to consider the historical context, dating back to the 1930s when strong opposition to government intervention led to the repeal of the Sheppard-Towner Act in 1929. Expectations of resistance to comprehensive reform were well-founded.

Clinton's reform ventured into potentially disruptive territory without fully recognizing the inherent risks or prioritizing them adequately. The ambitious nature of the reform, while addressing vital issues, raises concerns about whether its architects fully understood the potential consequences and considered the complexities and challenges associated with such a significant healthcare sector overhaul.

President Clinton's confidence in pursuing healthcare reform becomes more understandable when viewed against the backdrop of remarkable economic growth during his tenure. The 1990s witnessed substantial economic expansion, famously known as the 'dot-com boom,' characterized by robust GDP growth and declining unemployment rates. Moreover, from a political standpoint, President Clinton enjoyed substantial support in Congress, a pivotal factor enabling him to advocate for ambitious healthcare reforms.

The Affordable Care Act (ACA): An In-Depth Analysis

Finally, after more than 40 years, a new health insurance reform bill passed in the US. Republicans would call it "Obamacare" to express their opposition to the bill. However, the official name of it is the Affordable Care Act. Another interesting point to mention here is that some Republicans expressed their dislike for Obamacare but favored ACA. Also, the Triumph reform, which was not passed, actually retained the main framework of ACA.

Understanding the ACA's Key Provisions

If we take a closer look at the ACA reform plan, we can observe that it circumvents Employer-Sponsored Insurance(ESI) as a more cost-effective option for those without employment and employers in small businesses. The ACA's primary objectives include expanding health insurance coverage and reaching out to those who do not qualify for Medicaid, also protecting individuals with pre-existing conditions.

ACA introduced several transformative provisions to reshape healthcare in the United States. It mandated that most Americans obtain health insurance, while implementing new regulations on insurers to ensure fair pricing based on factors like age, location, family composition, and tobacco use. The ACA also eliminated coverage caps, outlined ten essential health benefits for all plans, expanded Medicaid, and increased funding for the Children's Health Insurance Program (CHIP). Additionally, it addressed prescription drug coverage by closing the Part D coverage gap and provided immediate relief to those with pre-existing conditions through a national high-risk pool. To foster competition, the ACA permitted the sale of insurance across state lines, which we would discuss in next section.*(Summary of the Affordable Care Act, KFF)*

The Legislative Journey of the ACA

In earlier sections, we discussed the challenges of implementing healthcare reform in the United States. Now, let us explore how the ACA was able to become a reality.

It is not hard to imagine that there would be numerous obstacles to passing such a significant bill. Apart from Republican criticism, there is also division within the Democratic party on certain aspects of the bill. Additionally, the long and complex legislative process itself posed a formidable challenge (*How They Did It*). Dartmouth even published a study criticizing the ACA, suggesting that one-third of the expenses would be wasteful, with no improvement in the quality of care for people.

The ACA's successful passage was influenced by several interconnected factors. The turning point is the precedent set by healthcare reform in Massachusetts also contributed to a changing public opinion landscape. Meanwhile, President Obama made it a top priority during his term, mobilizing key figures to support its passage. Noteworthy, compromises were made throughout the legislative process to gain broader support.

Strategic Insights from the ACA's Passage

Does the content of ACA looks familiar? Yes, the main goals were also what Clinton's bill proposed. Or we can say Obamacare was influenced by Clinton's Bill.

Clinton's Plan is so disruptive, however, it does not prioritize it; on the contrary, Obamacare is building out, the team also prioritize to the first place. With this contradiction, it better to understand why the former failed to pass and the latter came into reality.

I would also contend that the success of Obamacare can be attributed, in part, to a stroke of luck. The endeavor to reform the healthcare system had been attempted and failed on numerous occasions. Consequently, individuals had accumulated valuable experience, and, somewhat peculiarly, healthcare reform had evolved into an aspiration or dream for certain key figures within the ACA.

Some insights we can learn from ACA include its mandates on individuals, which broaden the risk pool and reduce healthcare costs by mandating most Americans to have health insurance or face a tax penalty. (*Summary of the Affordable Care Act, KFF*) Additionally, with the

creation of health insurance marketplaces, it made the companies competitive, thus making the premium more affordable. Furthermore, the expansion of Medicaid in ACA is also noteworthy, as it aimed to involve uninsured people. Moreover, it extends the age of young people who can enjoy parents' insurance. Lastly, it establishes the Independent Payment Advisory Board, with specialists helping people enroll in ACA marketplace plans.

Evaluating the Effectiveness of the ACA

It's been over 10 years since ACA passed. How is it functioning to most people? I would like to start with the advantages that it brought to people.

Foremost, the ACA significantly increased coverage. According to *"The Uninsured and the ACA,"* nearly 20 million more people had coverage in 2016 than before the ACA was passed. The rate also hit a record low of 9% from 16%. This increase in coverage is a substantial contribution of the ACA, considering the difficulties people face when they are sick but lack insurance. It increased coverage among low-income individuals, since insurance became more affordable by expanding Medicaid and subsidizing premiums. Meanwhile, following the implementation of the ACA, the gap in uninsured individuals has narrowed, and credit is due to the expansion of public plans, subsidies, and the establishment of the advisory board.

ACA also worthy to highlight that most People in ACA plan are satisfied about it (*Americans' Experiences with ACA*). Although ACA is aiming to only 15% people, which the number is not impressive, it cannot be denied the significance who benefits from it. It eliminated the barrier for individuals with preexisting conditions who were typically rejected by insurance companies before the plan. Besides, the prescription drug cost less since it narrows the do not hole in part D of Medicare. Since 2010, almost 12 million Medicare beneficiaries have collectively saved more than \$26 billion on prescription medications (*CMS*). Women also benefit from ACA, it prohibits discriminatory premium pricing based on gender.

ACA brought a lot to health system; however there are several hole are not solved after 2010. The significant problem after ACA is the underinsured people. When talking about underinsured, we are saying the group of people facing to high deductibles, copayments, or out-of-pocket expense.

Based on the KFF survey on uninsured individuals, the uninsured rate among non-elderly Americans saw a decline from 2010 to 2016. However, this positive trend did not continue, and in 2022, it experienced a slight increase of 0.8%. One of the reasons could be the new government did not push the enrollment of ACA anymore. Leading the coverage rate still an issue in the system, which also left the problem of coverage gap. Additionally, the question of health insurance for undocumented individuals remains an unresolved concern, further contributing to the complexity of the issue.

Another pertinent issue lies in businesses' adeptness at identifying system loopholes. As an example, certain companies may strategically reduce employee working hours to circumvent the requirement of providing healthcare coverage. (Healthline).

Also some topics are also mentioned in the debate of ACA. For instance, selling insurance across state line. The status quo is that for a lot states, there is only one big insurance in that state after the regulation. Meanwhile, ACA raised the premiums for young and more health individuals, however, it is about trade-off. Also, as mentioned earlier, while women have benefited from the ACA, it's important to note that the law also prohibited abortion coverage, a topic that has caused controversy.

In summarizing the ACA, it represents a largely positive reform in the healthcare marketplace. Despite political debates, such as concerns from some Democrats about tax cuts for the wealthy or the creation of winners and losers, these issues are not the primary focus of the ACA. The ACA has significantly expanded coverage and improved affordability for low-income individuals, marking a substantial achievement compared to the previous landscape.

One aspect I wish the ACA had addressed from the outset is drug price regulation and a potentially more disruptive approach. However, it's possible that these omissions contributed to its successful passage.

Shaping the Future of Healthcare Reform

Dilemmas of Status Quo

Analyzing historical events and political choices elucidates the present healthcare system. Let's now delve into the initial question posited in this essay. Primarily, it's crucial to acknowledge that the roots of the current system trace back not merely 30 years, but as far as the 1930s. In previous decades, there were opportunities to reform healthcare for the better (*Hacker*). However, these opportunities were missed, leading to a path dependence. Path dependence refers to the difficulty in altering direction once a policy is entrenched within an institution—it becomes arduous to enact modifications or reforms once a policy has become institutionalized.

To grasp the intricacies of the U.S. healthcare system, we need to consider a few key elements. First off, private insurance giants have held a firm grip on the market for decades, leveraging financial might to shape policies through robust lobbying. Shifting to public opinion, it's a powerful force influencing healthcare policies, easily swayed by emotional appeals and polarized narratives. And when we dive into healthcare reform, we run into the hurdles of agency, moral hazard, and adverse selection—these terms might sound complicated, but they boil down to the fact that the system's information isn't balanced, and this affects how we think about introducing a public option. Navigating these complexities calls for a clear understanding and thoughtful policy solutions.

Given the challenges in healthcare reform, I recommend exploring specific directions for improvement.

Cost Containment

High medical costs are undeniably one of the foremost challenges in healthcare, and the demand for federal regulation of medical care prices by two-thirds of Americans reflects the urgency of this issue. Examining healthcare expenditures, we find that while private insurance covers a majority of Americans, the overall cost of private insurance surpasses that of public healthcare. This is partly due to the comprehensive coverage provided by public programs, particularly for the elderly who tend to require more hospital care. To address these soaring costs, the first step towards containment could begin with the public program. With federal funding at its core, the government could wield greater negotiating power and institute robust cost-monitoring mechanisms (*The Hamilton Project*).

Turning to the expenditure categories, both private and public healthcare allocate the highest sums to hospital care and physician costs. Targeting these top-spending areas is imperative for cost control. High hospital costs can be attributed to administrative expenses, corporate profits, overcharging, and excessive use of costly medical technology (*Harvard*). An effective strategy involves curbing the overuse of expensive medical equipment, as has been successfully implemented in Canada (*Harvard*). Redirecting funds towards preventive care can also yield long-term cost reductions. However, increasing copayments for expensive technologies may pose a financial burden on those who genuinely need them. While shared-savings programs are a popular proposal, they must be implemented cautiously to avoid administrative complexities and potential degradation of care quality.

The issue of physician costs is closely tied to prescription drug prices. Unlike other countries where governments partially control drug and healthcare prices, the U.S. relies on market forces, leading to escalating costs. We should remember the resistance encountered in attempting to reform drug prices, as evidenced by recent legislation failures.

A potential approach involves gradually reallocating profits from pharmaceutical companies, allowing stakeholders to adapt over time while safeguarding their main interests to mitigate resistance to reform. Second, engage medium-sized companies and collaborate with them by offering favorable policies to enable them to compete with industry giants, fostering competition to lower drug prices. Third, promote pricing transparency by mandating companies to disclose their cost details and pricing rationale. One challenge in pricing lies in the difficulty of proving the effectiveness of certain medications, which may lead to fraudulent practices by companies. To address this, I propose incorporating more medical professionals into the regulatory team. Additionally, after 2-3 years of using a particular medication, if fraudulent activity is proven, substantial high penalties can be imposed on the company. I believe that leveraging government power can play a key role in facilitating a healthier and more competitive market.

Simultaneously, addressing the information asymmetry problem is crucial to gain public trust during reform. Communicating the benefits of change, including improved service quality and reduced out-of-pocket expenses, is essential to allay concerns and encourage acceptance of healthcare reform.

The Future of ESI

When talking the healthcare coverage today, it's imperative to address the reform within ESI. One key area of concern lies in the potential for discrimination based on the size of companies. Larger corporations tend to reap more significant benefits from ESI, creating an inherent disparity. Furthermore, as we consider varying profit levels, insurance companies often shift the risk burden from themselves onto these companies.

While I may not have a concrete reform proposal for ESI at this moment, it's crucial to underscore that without reform in this domain, achieving comprehensive improvements in

healthcare may prove to be an uphill battle. The equitable evolution of ESI is critical for the improvement of our healthcare system.

Right-site care

Right-site care can cut wait times for appointments with network doctors, offering timely access to treatment at the most suitable healthcare facility based on specific medical needs, improving overall efficiency.

Extracurricular Reference

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